

Pre-Denial Medical Necessity Dossier

Lumbar spinal fusion (L4–L5) prior authorization — physician-authored medical-necessity argument submitted before the payer decides, organized around the anticipated criteria pathway.

Specimen only. Fictional case. No PHI. No real patient, provider, or payer. This memo demonstrates Clinovian's clinical escalation format for denial teams. It is not legal advice, not a treating-physician opinion, and not a binding U.S. medical determination. Proprietary criteria content has been generalized; actual deliverables reference the applicable licensed criteria framework as documented in the case record. Clinical escalation methodology led by an MBBS physician with payer-side utilization-management review experience; clinical reasoning support only, not a licensed clinical opinion.

SECTION 01

Case Snapshot

REQUESTED SERVICE	Lumbar spinal fusion, L4–L5 — posterior interbody fusion with instrumentation
PLAN TYPE	Commercial EPO
CURRENT STATUS	Prior authorization pending — payer has not yet issued a determination
DOSSIER PURPOSE	Proactive physician-authored medical-necessity argument submitted with the prior-auth request to preempt denial

SECTION 02

Why a Pre-Denial Dossier

Most prior-authorization denials for surgical procedures are issued on the basis of incomplete or generically submitted clinical information. The treating surgeon submits operative notes and chart summaries; the payer's utilization reviewer evaluates against a criteria pathway that requires evidence in a specific structure. When the required evidence exists in the record but is not organized in the way the reviewer evaluates it, the denial is issued not because the case lacks merit — but because the submission did not anticipate the criteria pathway.

The Pre-Denial Dossier maps the patient's clinical evidence to the anticipated criteria pathway *before* the payer makes the determination — so the reviewer's first look is at a complete, criteria-organized submission.

SECTION 03

Anticipated Payer Criteria Pathway

For lumbar spinal fusion, commercial payer criteria frameworks typically require documented evidence across four domains before authorizing the procedure:

CRITERIA DOMAIN	WHAT THE REVIEWER LOOKS FOR
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Failed conservative therapy	Documented trial of conservative measures (physical therapy, pain management, injections) over a specified duration — typically 6–12 weeks minimum — with documented failure to improve.
Structural pathology correlation	Imaging findings (MRI, CT) that correlate with the patient's symptoms and physical examination. The imaging must show a structural problem at the level proposed for surgery.
Functional impairment	Documented functional limitations — not just pain reports — that affect activities of daily living, work capacity, or mobility, and that have failed to respond to conservative measures.
Surgical appropriateness	Evidence that the proposed surgical intervention addresses the documented pathology, that less invasive surgical options have been considered, and that the patient's overall health status supports the procedure.

SECTION 04

Clinical Evidence Mapped to Criteria

Failed conservative therapy — 14 months documented:

Physical therapy: Two separate courses (8 weeks each) at two different facilities, spanning 14 months total. PT discharge summary from the second course documents: "Patient demonstrates minimal functional improvement. Continues to report radicular symptoms limiting ambulation to less than one block. Recommends surgical consultation." Epidural steroid injections: Three fluoroscopy-guided L4–L5

transforaminal epidural injections over 10 months. Pain diary documents temporary relief (2–3 weeks per injection) with return to baseline radicular pain. Medications: Sequential trial of NSAIDs (naproxen 6 weeks, meloxicam 8 weeks), gabapentin (titrated to 1800mg/day, 12 weeks, discontinued for side effects), duloxetine (60mg/day, 8 weeks, insufficient relief).

Structural pathology correlation:

MRI lumbar spine (3 months prior): Grade II spondylolisthesis at L4–L5 with bilateral foraminal stenosis. Left L5 nerve root compression documented with foraminal height reduction to 4mm (normal >7mm). Disc desiccation and loss of height at L4–L5 with posterior annular tear. Findings correlate with the patient's left-sided L5 radiculopathy — dermatomal pain distribution matches the level of compression.

Functional impairment:

Oswestry Disability Index: 58% (severe disability). Ambulation limited to less than one block with assistive device. Unable to sit for more than 20 minutes. Unable to perform job functions (warehouse work requiring lifting, standing, and walking). On modified work duty for 8 months; employer has indicated inability to continue accommodations beyond 60 days.

Surgical appropriateness:

Spine surgery consultation documents consideration of both decompression alone and decompression with fusion. Given Grade II spondylolisthesis with instability on flexion-extension films (4mm dynamic translation), decompression alone is contraindicated due to risk of progressive instability. Fusion with instrumentation is the appropriate intervention. Patient is 52 years old, BMI 28, non-smoker, no surgical contraindications, medically optimized for procedure.

Pre-Emptying Common Denial Rationales

POTENTIAL PAYER OBJECTION

"Conservative therapy was not exhausted — additional physical therapy or injection courses should be attempted."

PRE - E M P T I O N

Patient completed 14 months of documented conservative therapy across multiple modalities. Two PT courses with two different providers, three epidural injections, and four medication trials — all documented with specific dates, durations, and outcomes. The criteria threshold is a reasonable trial, not indefinite repetition of failed interventions.

POTENTIAL PAYER OBJECTION

"Fusion is not indicated — decompression alone is less invasive."

PRE - E M P T I O N

Decompression alone is contraindicated in the setting of Grade II spondylolisthesis with documented dynamic instability on flexion-extension films. The surgeon's note explicitly addresses this — decompression without fusion risks progressive slip and clinical deterioration. This is a clinical determination, not a preference.

SECTION 06

Documentation Gaps

Gap 1 — Flexion-extension films not included in prior-auth packet: The surgeon references dynamic instability on flexion-extension radiographs in the operative plan,

but the images and radiology report were not included in the submitted prior-auth documentation. These must be attached — they are the specific evidence that differentiates fusion from decompression-alone.

Gap 2 — PT discharge summary from first course: Only the second PT discharge summary is in the submitted records. The first course documentation should be included to demonstrate the full 14-month conservative-therapy timeline.

No clinical gaps. The clinical evidence is strong. The gaps are submission gaps — evidence that exists but was not included in the prior-auth packet.

SECTION 07

Submission Recommendation

Submit the dossier with the prior-authorization request. Attach: (1) this dossier, (2) lumbar MRI with radiology report, (3) flexion-extension radiographs with radiology report, (4) both PT discharge summaries, (5) injection records with dates and outcomes, (6) medication trial timeline, (7) Oswestry Disability Index scoring sheet, (8) surgeon's operative plan with decompression-vs-fusion rationale.

By organizing the clinical evidence around the anticipated criteria pathway before the determination, the payer reviewer receives a submission that maps directly to their evaluation framework. The goal is to make approval the path of least resistance — not to create a case the reviewer must work to deny and then defend on appeal.

Prevention Note

For future similar cases: The two submission gaps identified in this dossier (missing flexion-extension films and incomplete PT documentation in the prior-auth packet) are operational, not clinical. The evidence existed — it wasn't submitted. A prior-authorization checklist for surgical cases that maps required documentation to anticipated criteria domains would prevent this class of denial entirely. The clinical case was never in doubt; only the submission was incomplete.

Forward-looking operational insight for the client's surgical scheduling and prior-auth teams.

SECTION 08

Internal Pattern Tags

PRE-DENIAL DOSSIER

ORTHOPEDIC: LUMBAR FUSION

FAILED CONSERVATIVE THERAPY

PAYER: COMMERCIAL EPO

STRENGTH: STRONG

Want this level of reasoning on your own cases?

START A 3-CASE EVALUATION

SEND A DE-IDENTIFIED CASE

CASE SUITABILITY

Clinovian

This specimen demonstrates the Pre-Denial Medical Necessity Dossier — a physician-authored argument submitted before the payer decides, mapping clinical evidence to the anticipated criteria pathway before the initial determination.

[Start a 3-case evaluation →](#) · [Send a de-identified case →](#)

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